

INDUSTRIAL COURT OF QUEENSLAND

Workers' Compensation and Rehabilitation Act 2003 - s. 561(1) - appeal against decision of industrial commission

Q-COMP v Glen Rowe (C/2009/6)

PRESIDENT HALL

12 May 2009

DECISION

Mr Glen Rowe was employed by an automotive dealership in Townsville for a period of 22 years. For the later 15 years of his employment, he was employed as the Sales Manager of a Hyundai franchise conducted by the dealership. His employment came to an end on 22 April 2008. Mr Rowe resigned following an incident at work on that day. In June of 2008 Mr Rowe made an application for benefits under the *Workers' Compensation and Rehabilitation Act 2003* (the Act) in respect of a psychological/psychiatric injury. The information sheet completed by Mr Rowe attributed the psychological/psychiatric injury to:

- (i) events of 22 April 2008;
- (ii) replacement of older staff with less experienced staff; and
- (iii) pressures of working with unsuitable staff and failure by management to support him over the difficulties he was encountering.

WorkCover accepted that Mr Rowe had suffered a "personal injury" within the meaning of s. 32(1) of the Act and that his employment had been a significant contributing factor to that "personal injury". However, WorkCover rejected his claim on the ground that the psychological/psychiatric injury was excluded from the statutory definition of "injury" by the operation of s. 32(5) of the Act. Mr Rowe sought a Statutory Review. By a decision dated 11 August 2008, Q-COMP confirmed the decision of WorkCover to reject the application for benefits. In short form, Q-COMP determined that the events of 22 April 2008 constituted management action taken but that the management action was not reasonable. The decision of WorkCover was confirmed because the "stressors" raised under the headings at (ii) and (iii) were found to involve reasonable management action taken in a reasonable way and because the psychological/psychiatric injury was found to have arisen out of or in the course of that reasonable management action. Mr Rowe appealed against that Review decision to the Queensland Industrial Relations Commission. By a decision of 6 February 2009 (now reported at 190 QGIG 93), the Commission allowed the appeal, set aside Q-COMP's decision and determined that Mr Rowe's claim was one for acceptance. From that decision Q-COMP now appeals to this Court.

As the Commissioner correctly recorded, the essence of Mr Rowe's case at first instance was founded on the opinion of a psychiatrist (Professor Basil James) that, as a result of the events of 22 April 2008, Mr Rowe had suffered "an acute aggravation of a major depressive disorder" which was properly to be described as an aggravation of a personal injury within s. 32(3)(b) of the Act. I hasten to add that in advancing that submission Senior Counsel for Mr Rowe did not impliedly concede that a "personal injury" within the meaning of s. 32(1) had arisen out of or in consequence of the "stressors" gathered together at headings (ii) and (iii). On the contrary, it was the submission of Junior Counsel for Mr Rowe that the "stressors" gathered together at (ii) and (iii) were but "irritants" which did not impede Mr Rowe's capacity to function in the workplace. The reference to "aggravation" and to s. 32(3)(b) of the Act arose because before any of the "stressors" identified at headings (i), (ii) and (iii) developed in the workplace, Mr Rowe had been diagnosed as suffering from a depressive disorder. On the evidence of his treating general practitioner (Dr Dignam) who gave evidence at first instance Mr Rowe had been taking prozac for two years prior to the general practitioner's first consultation with him on 26 February 2002. The general practitioner had continued to prescribe that medication for Mr Rowe in the period between February 2002 and April 2008. It was that existing medical condition which was said to have been "aggravated" by the events of 22 April 2008.

The case developed by Q-COMP at first instance was that the events of 22 April 2008 and their sequela could not be separated out and treated as discrete from the impact of the "stressors" identified at (ii) and (iii). The primary submission appears to have been that the "aggravation" which flowed from the events of 22 April 2008 occurred only because the stressors at (ii) and (iii) had undermined Mr Rowe's psychological condition to the point at which he could not cope with the events that occurred on 22 April 2008. At least on the Appeal to the Court there was a secondary submission that the "stressors" at headings (ii) and (iii) were inextricably linked with the consequences of the events of 22 April 2008 because the "aggravation" of Mr Rowe's medical condition was not contributed to by what happened but was contributed by Mr Rowe's flawed perception of what happened. The perception was said to be attributable to the "stressors" gathered together at headings at (ii) and (iii).

In the event, the Commissioner accepted the first of the arguments advanced by Q-COMP. Materially, the Commissioner said:

"I have concluded that based on an assessment of the entirety of the evidence that multiple stressors were present and those were the ones identified in his application as supplemented by his oral evidence. Professor James' opinion

that Mr Rowe had employed coping mechanisms to deal with the stressors identified under headings (ii) and (iii) above is consistent with the evidence given by Mr Rowe and also by Mr Pickering who referred to the sales achievements of Mr Rowe's team over the relevant period. For reasons explained above, I accept that these stressors arose out of, or in the course of, reasonable management action taken in a reasonable way. When the events of 22 April 2008 occurred Mr Rowe's coping mechanisms had been exhausted and his final decompensation occurred. These events were profound and significant in their impact on Mr Rowe and did not arise out of, or in the course of, reasonable management action taken in a reasonable way.". [190 QGIG 93 at 95]

In my view, the Commissioner's conclusion was plainly open to Her Honour. Indeed, any other conclusion would have been difficult to reconcile with the medical evidence.

A convenient starting point is Dr James Report of 13 November 2008. (There was an earlier draft report of 6 November 2008). Having referred to the events of 22 April 2008 as triggering "depressive symptomology" and an "overt depressive illness", Dr James concluded:

"It is my further opinion that had Mr Rowe not experienced the work 'stressors'* he describes, he would not have had his more recent depressive illness.". [*I interpolate that the "stressors" were the "stressors" at headings (ii) and (iii).]

In the course of examination-in-chief, Dr James takes the matter further asserting that the "stressors" at headings (ii) and (iii) had exhausted Mr Rowe's capacity to cope with stress. Materially Dr James said:

"... but I think just one other thing that may be worth saying in this regard, and this is very fundamental to the whole field of stress research, that stress has its effect partly through its intensity the magnitude of an event, but it also has an effect through its duration and that people can cope as is called the so called stage of resistance, they can cope often for quite a long period of time with stressors, but they reach a point where those coping strategies and mechanisms are exhausted and that's been called the stage of exhaustion and at that point illness can supervene. How one becomes ill is to some extent a matter of past experience, to some extent a matter of genetics. Some people may get a peptic ulcer, some people get a psychiatric disorder such as depression. I think it's important to say about depression in this regard that the kind of depression that people might get who are predisposed, is not simply a psychological experience of being dejected and sad but it's a fundamental, biological change that can happen quite dramatically and has a very clear profile of symptoms that is well defined in the DSM4.".

Shortly thereafter, and following a discussion of psychological baggage acquired by Mr Rowe in his childhood, there was the following exchange:

"Now, can a person take those qualities and live a normal life, even in the face of quite strong stressors? -- Yes, they can. That is what I referred to a little while ago, as the stage of resistance, the adaptational processes that we all have. A particular type of coping. And, they can last sometimes for quite long periods of time. But either under more intense stress or if it goes on just too long that a person may succumb into some form of illness.".

In re-examination Dr James stressed that he was not asserting that the "stressors" at headings (i), (ii) and (iii) were cumulative, nor that Mr Rowe broke down over time. The nexus between the "stressors" at headings (ii) and (iii) and the "sudden break-down" appears from the following exchange:

"So the final psychological decompensation, what does that mean? -- It means the point at which Mr Rowe's coping mechanisms that I referred to earlier as the stage of resistance, could no longer cope and have been exhausted and illness supervened.

And as I understand it, he's coping mechanisms, they break down over a period of time, do they because of all his stressors that he's nominated? -- I don't think they break down over time. I mean this seemed to me, and it often is, quite an abrupt breakdown. The graph which is the exempla of the stress research shows a parallel line which is the stage of resistance and then a certain decompensation.

But he's spoken about this intensifying, bullying and management style so it would all be building up and coming to a head, wouldn't it? -- Yes, they would all be making demands on his coping systems, yes.".

The evidence of Mr Rowe's treating general practitioner (Dr Dignam) was that although on his six monthly visits over the period of exposure to the "stressors" at headings (ii) and (iii) she had made enquiries about symptoms of depression, he had consistently indicated:

"... that his depression seemed to be under better control than it had been previously and his activities were normal and there didn't seem to be any recurrence of any major depression; that seemed to be under control.".

However, at a consultation on 24 April 2008, i.e. two days after the events of 22 April 2008, Mr Rowe revisited history and showing not a little prescience ventured a self-diagnosis not unlike that developed by Dr James. The relevant passage is:

"Now, there's an entry there on the 24th of April 2008? -- Yes.

Now, can you explain to the Commission the matters that were discussed at the attendance? -- Yes. At that consultation, he'd - he had lost a little bit of weight. His blood pressure had reduced but he told me that he had - things had come at work to a bit of a head and just on one particular day which he said was 22nd of April, two days prior to this visit, that he's just lost the plot and had suddenly realised he didn't want to take it any more, that he'd had enough, and when he was talking about that he said that in retrospect with the symptoms once he'd had that outburst he looked back at what had been happening and realised that he probably hadn't been recognising symptoms where he was maybe getting a bit more anxious at times or maybe not feeling as well as he might do, but each time he thought, oh, it'll be all right and he sort of put it to the back of his mind, and, instead of acting on some of those things, he just carried on and things had built up. His frustrations had built up to the point where he just lot [sic] the plot and felt he couldn't continuing working any more.

Did he mention what those frustrations were? -- Yes. He mentioned that there had been a change in management at work and he was put in a position where he was required to pull some other co-workers or subordinates into line but he felt that, really, that was a difficult job to do because he agreed with what their grievances were and what their problems were and he felt that the way that he was asked to do this was not what he would normally have done himself; that he might've had a bit more leniency or a bit more of an arbitration-type of approach or a conciliatory-type of approach rather than to just say, 'Pull up your socks, you can't do that any more', type approach, and it didn't sit comfortably with him to do that when he felt like these people were colleagues and he just felt it was all a bit too much; that he didn't have the recognition that he'd had with the previous bosses where he'd been sometimes consulted about policy and direction of the company, and the new people taking over didn't use him in that way or didn't relate to him in that way.

Yes? -- And he felt it was just being difficult and he wasn't getting any appreciation for that.

Yes. And did he say anything about whether he understood or - understood that the employees needed to be disciplined? -- Yes, he understood that there was a necessary - that something happen about this, that action be taken, but he wasn't a hundred per cent in favour of the action that he was asked to take. He might've gone about it a different way.

Yes. And when he - you have a note there about aggressive - sorry - 'was having problems with people who were aggressive and disruptive'? -- Yes.

Did he mention who those people were? -- I understood - I didn't ask him specifically there, but I assumed it to be the people that were the staff that he was - because he hadn't been able to choose his own team, that some of the people that were on the team weren't going along with team policy or weren't doing things the way they were expected to do or perhaps were putting in their own opinions when that wasn't welcome or wasn't necessary or was inappropriate, and that they were doing that in an aggressive manner and in a manner that was contributing to the team not functioning properly or the workplace not functioning properly. But I didn't ask him specifically any more about that."

In circumstances in which the history given to each of Dr James and Dr Dignam by Mr Rowe largely accords with his evidence to the Commission it is not surprising that the Commission chose to adopt a view consistent with the medical evidence. (The other health professional who gave evidence, a psychologist viz., Dr Promnitz, does not contribute to the debate of the relative point.)

Understandably, the Commission did not deal with Q-COMP's second argument, viz., that the effect of the "stressors" at headings (ii) and (iii) had been to flaw Mr Rowe's perception of events of 22 April 2008. Since the matter has been traversed on the Appeal, it is appropriate to make some brief remarks about it. The first comment is, the argument seems to me not to be consistent with the medical evidence. Each of the health professionals who gave evidence, viz., Dr James, Dr Dignam and Dr Promnitz, attributed Mr Rowe's perception of the "stressors" at the headings at (ii) and (iii) to the moral values which he had developed as a result of childhood experience and experience in the army and to Mr Rowe's relatively rigid personality. Doubtless, those values and what Dr Dignam described as his "stereotype" way of looking at life influenced the impact upon him of the events of 22 April 2008. Indeed, in cross-examination Dr James seems to have explicitly taken that point. However, that is a very different thing from an assertion that the "stressors" at headings (ii) and (iii) had effected his perceptions and that proposition seems not to have been explicitly developed with any of the health professionals. The second comment is that, but for s. 32(5) of the Act, at least at a conceptual level, a psychological injury arising out of interaction between a worker's personality and the work environment would be compensable, compare *Flinders Power Operating Services Pty Ltd (formerly NRG Flinders Operating Services Pty Ltd) v Amato* [2007] SAWCT 33 at paras 125 and 126. The third comment is that there is an

unresolved question whether flawed perception is excluded only in the case of "action being taken against the worker" (s. 32(5)(b)) or whether s. 32(5)(a) and s. 32(5)(b) of the Act are parallel bars to inclusion within the statutory definition of industry, neither one of which truncates or limits the operation of the other. It is preferable to leave that matter for full argument in a case in which the point matters.

The reason that the interrelationship between s. 32(5)(a) and s. 32(5)(b) does not matter in this case is that by the passage previously extracted, the Commissioner actually decided the matter in favour of Q-COMP for reasons unrelated to notions of flawed perception. I reproduce the passage again (emphasis added):

"I have concluded that based on an assessment of the entirety of the evidence that multiple stressors were present and those were the ones identified in his application as supplemented by his oral evidence. Professor James' opinion that Mr Rowe had employed coping mechanisms to deal with the stressors identified under headings (ii) and (iii) above is consistent with the evidence given by Mr Rowe and also by Mr Pickering who referred to the sales achievements of Mr Rowe's team over the relevant period. For reasons explained above, I accept that these stressors arose out of, or in the course of, reasonable management action taken in a reasonable way. When the events of 22 April 2008 occurred Mr Rowe's coping mechanisms had been exhausted and his final decompensation occurred. These events were profound and significant in their impact on Mr Rowe and did not arise out of, or in the course of, reasonable management action taken in a reasonable way." (190 QGIG 93 at 95)

Lest there be doubt about what was being found in an earlier passage (again at 190 QGIG 93 at 95) the Commissioner concluded:

"Despite the valiant attempt made by Mr Fleming, QC the evidence from Mr Rowe and Professor James is that multiple stressors were present in the aggravation of this injury. It is true that Professor James' opinion is that Mr Rowe was coping with the stressors falling under the headings (ii) and (iii) but that does not mean that the stressors were not present or were not causing Mr Rowe some distress. Firstly, the submission that the mere fact that Mr Rowe was coping with the multiple stressors shows the presence of more than one stressor and further, Professor James said in evidence that all three of the main factors specified by Mr Rowe in his workers' compensation application contributed to his 'load of stress'. Further, that two of the stressors have been shown to arise from perceptions does not diminish their existence."

On those findings Mr Rowe, who bore the onus had not barred the operation of s. 32(5)(a) of the Act. The very reason of this Appeal is, of course, that Mr Rowe succeeded at first instance. How it was that the Commissioner accepted Q-COMP's submission but allowed Mr Rowe's appeal.

"In recent times much debate has been had over when an injury is removed from the operation of s. 32(1) of the Act where multiple stressors are present, one or more of which have been touched by reasonable management action taken in a reasonable way. It has been the consistent view of Q-COMP in reliance of the decisions of President Hall in *Q-COMP v Education Queensland* (2005) 179 QGIG 491 and *Prizeman v Q-COMP* (2005) 180 QGIG 481 that even if one stressor is caught by reasonable management action that will be sufficient to remove the psychiatric/psychological disorder from the definition of injury in s. 32(1) of the Act. This view was repeated in the review decision against which this appeal is filed and submitted as part of this appeal hearing.

In *Q-COMP v Hohn* (2008) 187 QGIG 139 ('Hohn') the President refers to writings by Watson, the author of *Industrial Laws Queensland*, in discussing reasonable management action and in particular the circumstances in which an injury is withdrawn because of the reasonable management action exclusion in s. 32(5) of the Act. Given the President's endorsement of certain of the views expressed by Watson, it is useful to refer to his views in light of the decision in *Hohn*. After traversing the various decisions which have been central to this debate the following is written:

'So it can now be definitely said that if there are multiple events or stressors and only one or some of them arise out of or occur in the course of reasonable management action taken in a reasonable way then the claimant may succeed in the claim based on any that do not so arise or occur in the course of reasonable management action taken in a reasonable way.' at [71,015]."

In fairness to Q-COMP I should say that I cannot recollect an occasion on which (in this Court), Q-COMP has submitted that "... even if one stressor is caught by reasonable management action that will be sufficient to remove the psychiatric/psychological disorder from the definition of injury in s. 32(1) of the Act." Neither have I any recollection of accepting such a proposition. Indeed, such a proposition is not consistent with the decision in *Delaney v Q-COMP* (2005) 178 QGIG 197. In *Delaney v Q-COMP*, *ibid*, the claimant succeeded though the sixth stressor (not as was contended for Q-COMP in this Appeal the first stressor) was found to be but blemished management action. With respect to the Commissioner the decision in *Prizeman v Q-COMP* (2005) 180 QGIG 481, dealt with the converse proposition, viz., that if after eliminating all stressors which arose out of or in the course of reasonable management action reasonably taken, a stressor or stressors remained, the claimant was entitled to succeed. The earlier decision in *Q-COMP v Education Queensland* (2005) 179 QGIG 491 established the critical proposition that s. 32(5) of the Act is

concerned with withdrawing injuries from s. 34(1) of the Act and is not concerned with nominating the "stressors" which may be taken into account. The passage from the Industrial Laws of Queensland, is by way of commentary upon the decision of this Court in *Q-COMP v Hohn* (2008) 187 QGIG 139. It is made by way of comment upon the rejection of the view that so long as a psychological/psychiatric injury is "touched" by reasonable management action reasonably taken, it will be withdrawn from the statutory definition of "injury" by the operation of s. 32(5)(a) of the Act. The passage is plainly correct. It is the effect of the decision in *Q-COMP v Hohn*, *ibid*, that a claimant may succeed though some of the operative events or stressors arise out of or occur in the course of reasonable management action taken in a reasonable way. However, "may" cannot be read as "must": nor may the passage be read as asserting that an Appeal Body is at liberty to allow a claimant to succeed where at least one stressor does not "... arise or occur in the course of reasonable management action taken in a reasonable way". In all such cases, the Appeal Tribunal will be required to embark upon the enquiry whether the psychological/psychiatric injury arose out of or in the course of reasonable management action taken in a reasonable way. Here, that very enquiry was undertaken. The outcome was adverse to Mr Rowe. It follows that Mr Rowe does not have a recognisable claim.

I allow the Appeal. I set aside the decision of the Industrial Commission. In lieu thereof, I order that the Review decision of Q-COMP be confirmed.

Dated 12 May 2009.

D.R. HALL, President.

Appearances:

Mr G. Long SC and with him Mr S. Gray, directly instructed by the Appellant.

Mr K. Fleming QC and with him Ms L. Willson instructed by Dempseys Lawyers, for the Respondent.

Released: 12 May 2009

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